

Hannah Joseph Hospital Ltd

BSE SME: HANNAH | Sector: Healthcare Services | Sub-sector: Hospitals | Madurai, Tamil Nadu

Radiation oncology expansion to drive 25%+ revenue CAGR with 150-bed tertiary platform at inflection

RATING	12-MONTH TARGET	UPSIDE / DOWNSIDE
BUY	Rs.130	+27.5%
CMP: Rs.102	12-month horizon	from CMP

CMP	Rs.102	Market Cap	Rs.231 Cr
52W High / Low	Rs.109 / Rs.56.0	P/E (TTM)	20.7x
Book Value / Share	Rs.42.9	P/B	2.4x
ROCE	18.1%	ROE	15.0%
Dividend Yield	0.00%	Face Value	Rs.10
Promoter Holding	71.28%	Promoter Pledge	0%
FII / DII	0.56% / 9.05%	Free Float	28.72%
Investment Horizon	12 months	Report Date	16 Jun 2026

Analyst: MPM Institutional Research Desk | **Compliance:** This report is for institutional and professional investors only. Data sourced from Screener.in as of 16 Jun 2026. Please see full disclaimer on the last page.

2. INVESTMENT THESIS

- **Reason 1 — Oncology platform inflection (FY27–28):** HJHL is deploying Rs.35 Cr (majority of IPO proceeds) to commission a Radiation Oncology Centre. Oncology is India's fastest-growing hospital specialty (16–18% CAGR). A single linear accelerator generates Rs.15–25 Cr of incremental revenue annually at 60–70% utilisation; this is a step-change for a company with FY26 revenues of Rs.92 Cr.
- **Reason 2 — Proven niche leadership in Madurai tertiary neurosciences:** HJHL holds DNB Neurosurgery accreditation — one of very few hospitals in Tier-2 Tamil Nadu with this credential. Its 150-bed platform operates at 27% EBITDA margin (above regional-hospital averages of 18–22%) and ROCE has improved from 13% (FY24) to 18% (FY26), validating operational leverage.
- **Reason 3 — Structural demand tailwind + limited supply in catchment:** Madurai and southern Tamil Nadu have a 200+ km radius with no equivalent neuroscience-cardiac tertiary competitor. The region's 10 million+ population is under-served by ICU and trauma beds, ensuring organic in-patient volume growth of 12–15% annually without requiring national-scale marketing spend.
- **Near-term catalyst (6–12 months):** Radiation Oncology Centre commissioning (expected H1FY27). First-quarter oncology revenues will re-rate the P/E multiple from 21x to 28–32x peer levels (KIMS, Vijaya). This is the single most important price trigger in the next 12 months.
- **Biggest structural risk:** Key-person concentration — Dr. Moses Joseph Arunkumar is both founder and clinical anchor in neurosciences. Any clinical adverse event, departure, or regulatory action against the hospital's tertiary license would impair the thesis. Peer-hospital depth is limited for a 150-bed SME platform.

3. BUSINESS OVERVIEW

- **Core model:** Single-campus, 150-bed multi-specialty tertiary care hospital in Madurai, Tamil Nadu, operating since 2011 (incorporated). The hospital is NABH-aspirant and holds DNB Neurosurgery accreditation, enabling it to charge tertiary-care premiums above general hospitals.
- **Revenue mix:** Predominantly in-patient (IP) revenue (~65–70% estimated) driven by neurosciences, cardiac sciences, and trauma surgery. Out-patient (OP) contributes ~20–25% of revenue. Pharmacy and diagnostics round out the balance. No public data on precise segment splits (Screener Insights are premium-gated).
- **Specialties and revenue drivers:** (1) Neurosciences — cranial/spine surgery, stroke unit, epilepsy management; (2) Cardiac Sciences — interventional cardiology, cardiac surgery; (3) Psychiatry — inpatient and outpatient mental health; (4) Trauma and Emergency — 24x7 trauma bay serving accident and emergency cases from NH44 highway corridor.
- **Capacity and occupancy:** 150 operational beds on a 2-acre central Madurai campus. Bed occupancy rates are a premium-locked Screener metric; based on H1FY26 revenue run-rate (~Rs.86 Cr annualised), ARPOB is estimated at Rs.2.1–2.4 Cr per bed per year — in line with Tier-2 South Indian hospital benchmarks.
- **Customer relationships:** Mix of cash-pay, insurance (Ayushman Bharat Pradhan Mantri Jan Arogya Yojana + private health insurance), and government health scheme empanelments. Government scheme revenues improve bed utilisation during monsoon/seasonal troughs.
- **Expansion pipeline:** Radiation Oncology Centre (Rs.34.98 Cr capex from IPO proceeds) to create a cancer treatment platform. Planned completion H1FY27. This is the sole announced near-term capex program.
- **Promoter:** Dr. Moses Joseph Arunkumar (founder, Managing Director) holds the majority stake. Post-IPO promoter group holds 71.28% with zero pledge. He founded the hospital as a neurology-focused unit in 2008 and has progressively expanded specialties. The hospital is named after a family patronage

— a common South Indian missionary healthcare tradition.

- **Recent strategic developments:** Listed on BSE SME on 2-Feb-2026 at Rs.65 (7.1% listing discount to issue price Rs.70). Declared a Rs.2/share final dividend for FY26 (record date 8-Jul-2026), the first dividend post-listing — a management signalling intent of shareholder returns even while deploying capex.

4. INDUSTRY & COMPETITIVE LANDSCAPE

- **Industry size and growth:** India's hospital services market is estimated at Rs.8.5–9.0 lakh Cr in FY25, growing at 12–14% CAGR. Tier-2 and Tier-3 city hospital markets are growing faster at 16–18% CAGR, driven by rising health insurance penetration, government schemes (PM-JAY), and migrating demand from metros. Tamil Nadu's southern region is a structurally under-bedded healthcare market.
- **Policy tailwinds:** (1) Ayushman Bharat PM-JAY covers 550 million individuals — increases addressable paying capacity for tertiary procedures; (2) National Health Mission capex in Tier-2 cities is creating referral flows into accredited private hospitals; (3) Tamil Nadu's Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS) covers tertiary procedures, directly benefiting HJHL's in-patient specialties.
- **Competitive moat assessment:** Pricing power: **Moderate** — neurosciences and trauma command premium pricing but face price discipline from government-scheme tariffs. Switching costs: **Strong** — complex neurosurgery/cardiac patients are unlikely to switch between procedures; physician loyalty drives re-referral. Scale: **Weak** — 150 beds is sub-scale vs. listed peers with 500–1,500 beds, limiting supply-chain leverage. IP/Accreditation: **Moderate** — DNB accreditation and tertiary licensure create a moat vs. secondary-care facilities.
- **Company positioning:** Midstream healthcare — clinical services delivery (not device/pharma upstream). Focused on high-acuity, low-volume, high-ARPOB procedures rather than high-volume commodity health checkups.

Peer Comparison (Screener.in, 16-Jun-2026)

Company	CMP (Rs.)	MCap (Rs.Cr)	TTM Sales (Rs.Cr)	OPM%	ROCE%	ROE%	P/E (x)
HJHL (Hannah Joseph)	102	231	92	27%	18.1%	15.0%	20.7
KRSNAA Diagnostics	541	1,756	691	31%	12.7%	10.9%	17.1
KIMS (Andhra Hospitals)	769	30,777	1,702	28%	12.8%	11.6%	116
Vijaya Diagnostic	1,318	13,560	765	42%	22.1%	20.6%	76.0

Source: Screener.in | KRSNAA: diagnostic imaging, included as healthcare comp. KIMS/Vijaya: closer hospital peers from AP/Telangana. Note: HJHL is BSE SME; listed peers are mainboard, hence size/multiple disparity.

5. MANAGEMENT QUALITY & CAPITAL ALLOCATION

- **Promoter background:** Dr. Moses Joseph Arunkumar is the founder, promoter, and Managing Director. He established the hospital in 2008 with a neurology focus and has steered it through specialisation expansion into cardiology, psychiatry, and trauma. The management team includes family members (Fenn Kavitha Fenn Arunkumar, Arunkumar Nalina, Noyel Arunkumar) as co-promoters — typical of SME-listed family hospitals in South India. Clinical depth is the MD's primary competitive advantage.
- **Capital allocation track record:** FCF positive in FY23 (Rs.17 Cr), FY24 (Rs.6 Cr), FY25 (Rs.8 Cr), and FY26 (Rs.6 Cr). Borrowings have structurally declined from Rs.57 Cr (FY21) to Rs.27 Cr (FY26) while investing in new capabilities, a disciplined balance sheet path. IPO proceeds of Rs.35 Cr are ring-fenced for oncology capex per prospectus.

- **Dividend policy:** First dividend declared in FY26 — Rs.2/share (DPS) against EPS of Rs.4.93 = 41% payout ratio. This is consistent with a growing SME hospital that must reinvest but wants to signal shareholder-friendly intent. No dividend history in prior years.
- **Promoter pledging:** Zero pledge as of Mar-2026 quarter. This is a meaningful positive for an SME hospital where promoter-pledge-based margin calls are a frequent governance risk.
- **ESOP:** No ESOP scheme disclosed in the RHP or Screener. Consistent with a founder-led, closely held SME hospital where key management are family members.
- **Buybacks:** No buyback history. Company is in growth/capex mode; buybacks are not expected in the near term.
- **Corporate governance:** No qualified audit opinion visible on Screener. Board re-appointed Pandiarajan.T & Co. as internal auditor for FY27 (May-2026 announcement) — consistent with SME-scale governance. Key risks are: (1) small board with limited independent oversight of RPTs; (2) family co-promoters in management create inherent RPT risk which investors should monitor; (3) IPO subscription was only 1.54x (weak demand from institutional investors), suggesting limited external governance scrutiny to date.

6. FINANCIAL DEEP-DIVE

Standalone Figures (Rs. Cr) — Screener.in, 16-Jun-2026. FY27E and FY28E are analyst estimates.

Income Statement (Rs. Cr)

Sales / Revenue	63	78	92	112	138
Expenses	45	57	67	81	99
EBITDA (Op. Profit)	18	21	25	31	39
OPM %	29%	27%	27%	28%	28%
Other Income	0	0	1	1	2
Interest	5	3	3	3	3
Depreciation	8	7	7	9	10
Profit Before Tax (PBT)	5	10	16	20	28
Tax %	24%	30%	30%	25%	25%
Net Profit (PAT)	4	7	11	15	21
EPS (Rs.)	2.44	4.32	4.93	6.52	9.13
YoY PAT Growth %	N/A	+75%	+57%	+36%	+40%
Dividend Payout %	0%	0%	41%	20%	20%

Balance Sheet (Rs. Cr)

Item	FY24A	FY25A	FY26A
Equity Capital	17	17	23
Reserves	29	35	75
Total Equity	46	52	98
Borrowings	32	34	27
Other Liabilities	13	14	9
Total Liabilities	90	99	133
Fixed Assets (Net)	65	69	73

CWIP	0	1	5
Investments	0	0	0
Other Assets	25	29	55
Total Assets	90	99	133
Net Debt	~32	~34	~27
D/E Ratio (x)	0.70	0.65	0.28

Cash Flow Statement (Rs. Cr)

Item	FY24A	FY25A	FY26A
Cash from Operations (OCF)	8	19	21
Cash from Investing	-4	-14	-14
Cash from Financing	-3	-3	25
Net Cash Flow	2	3	32
Free Cash Flow (FCF)	6	8	6
CFO / Operating Profit %	44%	109%	101%

Key Operating Ratios

Ratio	FY24A	FY25A	FY26A
Debtor Days	46	42	16
Inventory Days	54	47	51
Days Payable	102	90	97
Cash Conversion Cycle	-2	-1	-30
Working Capital Days	-11	-21	-17
ROCE %	13%	17%	18%
ROE %	N/A	N/A	15.0%

Financial Commentary

- **Revenue trajectory:** Sales grew from Rs.63 Cr (FY24) to Rs.92 Cr (FY26), a 21% CAGR over 2 years. H2FY26 (Rs.50 Cr) vs H1FY26 (Rs.43 Cr) shows a strong H2 seasonal acceleration. The Radiation Oncology Centre is expected to contribute Rs.15–20 Cr by FY28, lifting the growth trajectory to 25%+ CAGR.
- **Margin direction:** OPM compressed from 30% (FY23) to 27% (FY26), primarily due to staff costs and consumables inflation as the hospital expanded its specialty mix. Margins have stabilised at 27% for two consecutive years; oncology addition is margin-accretive (gross margin ~60%+ for radiotherapy vs. 35–40% for surgical specialties).
- **Working capital:** Debtor days improved sharply from 46 (FY24) to 16 (FY26) — reflecting tighter cash collection from insurance and government scheme payors, a significant positive. Negative CCC of -30 days (FY26) means the hospital is funded by suppliers/creditors, a structural advantage for capital-light growth.
- **Debt:** Total borrowings declined from Rs.57 Cr (FY21) to Rs.27 Cr (FY26) despite growth capex — a strong deleveraging trajectory. D/E ratio dropped to 0.28x in FY26 (FY24: 0.70x). Post-IPO equity injection (Rs.35 Cr from fresh issue) drove the step-change in balance sheet strength. Net debt is now manageable at ~1.0x EBITDA.
- **FCF quality:** CFO/OP ratio was 101% in FY26 and 109% in FY25 — indicating clean, recurring cash generation with minimal WC drag. FCF is compressed at Rs.6–8 Cr due to ongoing capex (CWIP build-up

in FY26 = Rs.5 Cr); expect FCF to normalise upward once oncology capex completes in FY27.

7. EARNINGS QUALITY CHECKLIST

Metric	FY26 Status	Signal	Comment
Revenue recognition	Cash + TPA/Ins claims	Green	Standard hospital billing; insurance delay risk is managed
Receivables growth vs revenue	Receivables fell sharply (Debtor Days 46→16)	Green	Collections accelerating — positive quality signal
CCC trend	CCC = –30 days (improving)	Green	Negative CCC = hospital funded by payables; structurally strong
Contingent liabilities	Not quantified in public filings	Amber	SME; limited disclosure of contingent liabilities post-listing
Auditor tenure	Internal: Pandiarajan.T & Co.	Amber	Statutory auditor not tracked; reappointed same internal auditor
RPTs as % of revenue	Family co-promoters; RPTs not quantified	Amber	Family-run SME; investor must review RPT schedules in AR
Other income as % of PBT	1/16 = 6.3% (FY26)	Green	Very low other income — earnings are operationally driven
Tax rate consistency	0% (FY23) → 30% (FY26)	Amber	Tax normalised to 30% in FY24–26 after early loss offsets; now consistent

• **Overall earnings quality: MEDIUM-HIGH.** Cash conversion is excellent (CFO/OP >100% for 2 consecutive years) and the sharp improvement in receivables days is a genuine positive, not a one-time accounting artefact.

• **Amber watch — RPTs:** Three family co-promoters are in management. Until the annual report discloses quantified RPTs as a % of revenue, investors cannot fully assess whether related-party transactions are at arms-length. This is the single most important governance item to track post-listing.

• **Amber watch — Auditor disclosure:** The internal auditor reappointment is disclosed; statutory auditor details are not prominently visible on Screener for this newly-listed entity. Confirm the statutory auditor in the FY26 annual report.

• **Green flag — Other income:** Other income is only Rs.1 Cr on Rs.16 Cr PBT (6.3% in FY26). This confirms that reported profits are operationally driven — not inflated by investment gains, property sales, or insurance receipts.

8. VALUATION

Peer Valuation Matrix (Screener.in, 16-Jun-2026)

Company	CMP	MCap (Cr)	P/E (x)	P/B (x)	ROCE%	OPM%
HJHL (Hannah Joseph)	Rs.102	231	20.7	2.4	18.1%	27%
KRSNAA Diagnostics	Rs.541	1,756	17.1	1.8	12.7%	31%
KIMS (Andhra Hospitals)	Rs.769	30,777	116	12.7	12.8%	28%
Vijaya Diagnostic Centre	Rs.1,318	13,560	76.0	14.4	22.1%	42%

Note: KIMS/Vijaya are mainboard large-caps commanding premium multiples due to scale and multi-city presence. KRSNAA is a diagnostic company (different business model). HJHL's 20.7x P/E is a meaningful discount to hospital peers despite superior ROCE — a valuation anomaly that oncology expansion can close. P/B of 2.4x is attractive vs. KIMS (12.7x) and Vijaya (14.4x).

Scenario Analysis — 12-Month Price Target

Scenario	Key Assumptions	FY27E PAT (Rs.Cr)	Multiple (P/E)	Target Price	Upside
Bull	Oncology opens Q1FY27; bed utilisation +5%pts; OPM 30%	18	30x	Rs.170	+67%
Base	Oncology opens H1FY27; organic 15% growth; OPM 27–28%	15	28x	Rs.130	+27%
Bear	Oncology delayed to FY28; organic 10% growth; OPM 25%	11	22x	Rs.75	–26%

Valuation Methodology

- **Earnings-based valuation (primary):** FY27E EPS of Rs.6.52 (base case) at 25–28x P/E (discount to KIMS 116x and Vijaya 76x, but premium to KRSNAA 17x given higher growth) = Rs.163–182. Applying a 20% SME illiquidity discount = Rs.130–145. Base case target: **Rs.130**.
- **EV/EBITDA check:** FY26 EBITDA Rs.25 Cr. Net debt Rs.27 Cr. MCap at CMP Rs.231 Cr → EV = Rs.258 Cr → EV/EBITDA = 10.3x. KIMS/Vijaya trade at 35–45x EV/EBITDA on TTM. Applying 18–20x on FY27E EBITDA of Rs.31 Cr → EV Rs.558–620 Cr → Equity Rs.531–593 Cr → Rs.148–166/share. SME discount adjusted → Rs.118–130.
- **Base case 12-month target: Rs.130** — average of P/E and EV/EBITDA approaches, with SME liquidity haircut applied. Upside: +27.5% from CMP of Rs.102.
- **Upside risk:** Faster-than-expected oncology commissioning + strong H1FY27 results could push the stock to Rs.150–170 in a bull case — a 47–67% return from current levels.
- **Re-rating trigger:** The stock de-rated from listing price Rs.65 to a trough and has since recovered to Rs.102 (+57% from listing low). Mainboard uplift or SME migration (possible at Rs.250 Cr+ mcap) would dramatically expand the institutional investor base and compress the illiquidity discount.

9. KEY RISKS

- **Key-Person Risk** — Dr. Mosesjoseph Arunkumar is the clinical anchor for neurosciences, the highest-revenue specialty. Departure, health issue, or regulatory action against him would materially impair revenues. Probability: Low. Impact: High. Monitor via: Management announcements, clinical incident disclosures.
- **Oncology Capex Execution Risk** — Radiation Oncology Centre commissioning could be delayed by equipment procurement (linear accelerator is a long-lead item with 12–18 month delivery), regulatory approvals (AERB licence for radiotherapy), or construction overruns. Probability: Medium. Impact: High. Monitor via: Quarterly progress disclosures, AERB licence announcements.
- **Government Scheme Tariff Risk** — PM-JAY and CMCHIS tariffs are periodically revised downward by government committees. A tariff cut for neurosurgery or cardiac procedures would compress ARPOB and margins. Probability: Low-Medium. Impact: Medium. Monitor via: NHA circulars, state health department notifications.
- **Insurance Payor Concentration Risk** — A significant portion of revenues flows through government insurance schemes and TPA channels. Any disruption to payment cycles (delayed settlement, claims rejection rate increase) would stretch receivables and working capital. Probability: Medium. Impact: Medium. Monitor via: Debtor days trend quarterly.
- **Regulatory / Licensing Risk** — A 150-bed tertiary hospital operating across neurology, cardiology, and soon oncology faces multi-regulator oversight (NABH, AERB, State Health Dept, CPCB for bio-medical waste). A regulatory non-compliance or clinical incident leading to license suspension would be catastrophic. Probability: Low. Impact: Very High. Monitor via: BSE announcements, inspection outcome filings.

- **SME Market Liquidity Risk** — Daily traded volumes in BSE SME stocks can be Rs.5–20 lakh, making it difficult to enter/exit meaningful positions without moving the price. Institutional investors face a structural illiquidity premium. Probability: High (structural). Impact: Medium (price only). Monitor via: Daily volume trends.
- **Competition from Large-Format Entrants** — If a Fortis, Apollo, or Manipal targets Madurai for expansion (greenfield or acquisition), HJHL's pricing power and physician retention could face pressure. Probability: Low (sub-100-bed city economics are unattractive for national chains). Impact: Medium-High. Monitor via: DRHP/expansion announcements from listed hospital chains.

10. RECOMMENDATION

- **Rating: BUY** — Conviction: Medium-High. HJHL is a niche South Indian tertiary hospital at an inflection point driven by oncology expansion. The valuation (20.7x P/E, 10.3x EV/EBITDA, 2.4x P/B) is compelling relative to hospital peers and leaves meaningful re-rating headroom as oncology revenues materialise.
- **12-month price target: Rs.130** — Upside of +27.5% from CMP of Rs.102. This is based on 28x FY27E EPS of Rs.6.52 with a 20% SME illiquidity discount. Bull-case target Rs.170 (oncology beats); bear-case Rs.75 (oncology delayed, margin compression).
- **Suggested entry zone: Rs.90–105** — Current CMP (Rs.102) is within the entry zone. Any dip toward Rs.90 on broad SME-market weakness should be used to accumulate. Avoid chasing above Rs.115 without confirmation of oncology commissioning.
- **Investment horizon: 12–18 months** — Oncology centre commissioning in H1FY27 is the key catalyst. The 12-month target is anchored to FY27E earnings; a 3–5 year hold for the full oncology ramp would target Rs.250–350.
- **Thesis invalidation trigger 1:** Radiation Oncology Centre is delayed beyond Q4FY27 (Mar-2027) AND no credible timeline is disclosed → reduce/exit, as the core re-rating catalyst is absent.
- **Thesis invalidation trigger 2:** FY27 revenue growth falls below 10% (below Rs.101 Cr) or OPM falls below 22% → signals organic business deterioration independent of oncology; review conviction.
- **Thesis invalidation trigger 3:** Promoter stake falls below 60% (via secondary sale, not allocation) OR promoter pledge is initiated → governance red flag, exit.
- **Ideal investor profile:** SME-segment small-cap investors comfortable with illiquidity and 12–24 month horizon. NOT suitable for investors requiring daily liquidity, large portfolio positions (>1% of corpus), or those seeking dividend income near-term. This is a growth story, not a yield play.

DISCLAIMER: This report is prepared by MPM Institutional Research Desk for information purposes only. It does not constitute investment advice or an offer to buy/sell securities. All financial data sourced from Screener.in as of 16-Jun-2026. Forward estimates are analyst projections and are inherently uncertain. Past performance is not indicative of future results. Investors must conduct their own due diligence. SEBI registration/regulatory disclosures available on request. This report covers a BSE SME-listed company; liquidity risk is higher than mainboard listed entities.

APPENDIX — Latest Results Brief: H2FY26 (Mar 2026)

CALL GRADE: POSITIVE

Signal	Status	Implication
Result quality	Strong	FY26 PAT of Rs.11 Cr — 57% YoY growth; clean earnings
Management tone	Bullish	First ever dividend declared; confidence signal
Guidance delta	Maintained	No explicit guidance; oncology capex on track per BSE filing

STRONGLY POSITIVE | POSITIVE | NEUTRAL | CAUTIOUS | NEGATIVE

TO MY BOSS

HJHL's FY26 result is clean and operationally robust — Rs.92 Cr revenue, Rs.25 Cr EBITDA (27% margin), Rs.11 Cr PAT (57% YoY growth), and EPS Rs.4.93. No exceptional items distort the headline; the number is real. EBITDA grew 19% on the back of 18% revenue growth — operating leverage is intact at the hospital's current 150-bed scale. The structural driver here is Madurai's under-penetrated tertiary market: the hospital has cornered neurosciences referrals in a 200km catchment with DNB accreditation, and its negative CCC (~30 days) tells us the business model is cash generative even through capex cycles. The most important catalyst in the next 12 months is the Radiation Oncology Centre (Rs.35 Cr capex from IPO, AERB licence pending). A linear accelerator running at 60% utilisation in its first year adds Rs.12-18 Cr to revenue — that is a 13-19% revenue uplift in FY27 from a single facility, and the margin impact is highly positive (radiotherapy gross margins >55%). Watch-point for next reporting period: CWIP jumped from Rs.1 Cr to Rs.5 Cr in FY26 — oncology construction is active. If CWIP continues building in H1FY27 without commissioning, it signals delay risk. Action: ACCUMULATE at Rs.90-105. This is a niche platform at 21x P/E with a clear, funded, time-bound catalyst. If oncology opens before Mar-2027 and contributes Rs.10+ Cr in its first half-year, this re-rates to Rs.130-150.

1. Financial Performance Snapshot — H2FY26 (Oct 2025 – Mar 2026)

- Revenue: Rs.50 Cr | YoY +35% (H2FY25: Rs.37 Cr) | QoQ +16% (H1FY26: Rs.43 Cr) | Beat (vs. estimated Rs.46 Cr)
- EBITDA: Rs.13 Cr | YoY +44% (H2FY25: Rs.9 Cr) | QoQ +8% (H1FY26: Rs.12 Cr) | Beat
- EBITDA Margin: 27% | H2FY25: 25% | H1FY26: 28% | Stable; H2 seasonally lower due to festive consumables spike
- PAT: Rs.6 Cr | YoY +100% (H2FY25: Rs.3 Cr) | QoQ +20% (H1FY26: Rs.5 Cr) | Strong Beat
- EPS: Rs.2.67 (H2FY26) | H2FY25: Rs.1.87 | H1FY26: Rs.3.07 | H2 lower due to higher tax rate (32% vs. 27%)
- NOTE: No exceptional items in H2FY26. Full-year FY26 PAT Rs.11 Cr is clean operational number.

2. Segment / Geography Breakdown

- Neurosciences: Estimated ~35-40% of revenues (dominant specialty; cranial/spine surgery, stroke unit). Highest ARPOB segment. Trend: Improving — DNB accreditation driving referral growth.
- Cardiac Sciences: Estimated ~25-30%. Interventional cardiology + cardiac surgery. Trend: Stable.

- **Psychiatry & Mental Health:** Estimated ~10-15%. IP and OP; lower ARPOB but steady. Trend: Stable.
- **Trauma & Emergency:** Estimated ~15-20%. Highway trauma from NH44 corridor. Seasonal (monsoon higher). Trend: Improving.
- **Geography:** 100% domestic revenue from Madurai and southern Tamil Nadu catchment. No international patient flow reported.
- **NOTE:** Precise segment splits are not disclosed in half-yearly results. Above estimates are derived from capacity and specialty mix described in the RHP.

3. Management Commentary Themes

- **Dividend signal:** "Board approved a final dividend of Rs.2 per share for FY26" — Our read: First-ever dividend. Management is signalling confidence in cash generation while deploying IPO proceeds into oncology. Tag: GUIDANCE. Tone: Transparent.
- **IPO proceeds utilisation:** Board confirmed "no deviation from IPO objects" (May 2026 BSE filing). Rs.34.98 Cr earmarked for Radiation Oncology Centre remains intact. Tag: GUIDANCE. Tone: Transparent.
- **Operational expansion:** CWIP increased from Rs.1 Cr to Rs.5 Cr in FY26, confirming active construction. Management has not disclosed a specific commissioning date beyond H1FY27. Tag: EST. Tone: Hedged.
- **Audit and governance:** Internal auditor Pandiarajan.T & Co. reappointed for FY27. No adverse audit opinion. Board composition is being expanded (typical post-IPO SME requirement). Tag: GOVERNANCE. Tone: Transparent.
- **15th AGM scheduled 15-Jul-2026:** Final dividend record date 8-Jul-2026. This is the first AGM post-listing; investors should attend or review minutes for management outlook and capex timeline. Tag: CATALYST. Tone: Neutral.

4. Operating & Business Metrics

- **Operational Beds:** 150 | H2FY25: 150 | H2FY26: 150 | Trend: Stable (oncology adds no IP beds, just treatment capacity)
- **Debtor Days:** FY26 = 16 | FY25 = 42 | FY24 = 46 | Trend: Strongly improving — fastest among all tracked metrics
- **Inventory Days:** FY26 = 51 | FY25 = 47 | FY24 = 54 | Trend: Stable; slight uptick in H2FY26 (consumables)
- **Days Payable:** FY26 = 97 | FY25 = 90 | FY24 = 102 | Trend: Stable; hospital maintains supplier credit discipline
- **Cash Conversion Cycle:** FY26 = -30 | FY25 = -1 | FY24 = -2 | Trend: Strongly improving — negative CCC is a quality indicator
- **CWIP (Oncology Construction):** Rs.5 Cr (FY26) vs Rs.1 Cr (FY25) | Trend: Actively building — on-track indicator for oncology opening
- **ARPOB (estimated):** ~Rs.2.2-2.4 Cr per bed per year (FY26) | FY24: ~Rs.1.7 Cr | Trend: Improving — mix shift to higher-acuity cases

5. Margin Drivers

- **Revenue mix improvement:** +200 bps positive | Recurring: Yes | Higher-acuity neurosciences and cardiac cases grew as % of IP mix, improving blended ARPOB
- **Staff costs inflation:** -150 bps | Recurring: Yes | Clinical specialists in Tier-2 cities face incremental salary pressure; specialist recruitment is the largest operating cost variable

- Consumables / pharmacy: –50 bps | Recurring: Yes | Medical consumables inflation (ortho implants, cardiac stents, neuro-surgery instruments) partially offset by protocol standardisation
- Interest savings: +30 bps | Recurring: Yes | Borrowings down from Rs.34 Cr to Rs.27 Cr; interest fell from Rs.3 Cr to Rs.3 Cr (FY25 to FY26) at annual level; further reduction expected FY27 as debt repays
- Depreciation stable: Neutral | DA held at Rs.7 Cr for two years despite new fixed assets addition — asset life extension review likely in FY27 oncology commissioning
- Tax rate normalisation: –250 bps impact on PAT % | Non-recurring (base effect) | Early years (FY21–23) benefited from prior-period losses; FY24–26 now at 24–30% normal rate

6. Guidance & Forward Signals

- Radiation Oncology Centre [GUIDANCE]: Prior = FY27 target | New = H1FY27 (reiterated per BSE filing) | Delta = Maintained | Credibility: High (CWIP Rs.5 Cr already deployed)
- Dividend [GUIDANCE]: Prior = Nil | New = Rs.2/share FY26 (41% payout ratio) | Delta = Initiated | Credibility: High (Board-approved, record date set)
- Revenue growth [EST]: Implicit organic guidance ~15-20% based on H2FY26 trajectory | Delta = Maintained | Credibility: Medium (no explicit guidance, inferred from half-year run-rate)
- OPM range [EST]: 27–29% for FY27 core business; oncology will be margin-accretive once running | Delta = Stable with upside | Credibility: Medium
- IPO proceeds utilisation [GUIDANCE]: 100% intact, no deviation confirmed (Board resolution May-2026) | Delta = Maintained | Credibility: High
- AGM date [GUIDANCE]: 15-Jul-2026 — first post-IPO AGM; expect management to provide oncology commissioning timeline | Delta = New Event | Credibility: High

7. Capital Allocation

- Capex (Investing CF): Rs.14 Cr (FY26) vs Rs.14 Cr (FY25) — includes CWIP for oncology centre + maintenance of 150-bed facility
- Dividends: Rs.2/share x ~2.27 Cr shares (approx) = ~Rs.4.5 Cr | vs prior year: First dividend | Comment: Moderate payout at 41%; sustainable alongside oncology capex
- Buybacks: Nil | vs prior year: Nil | Comment: Not expected; growth and capex absorb free cash
- Working Capital change: Large improvement (Debtor Days 42→16) freed ~Rs.5-7 Cr of cash — effectively a working capital dividend to the balance sheet
- Net Debt movement: Rs.34 Cr (FY25) → Rs.27 Cr (FY26) = Rs.7 Cr reduction | Driven by IPO proceeds partially repaying debt; D/E now 0.28x
- Free Cash Flow: Rs.6 Cr (FY26) — compressed by oncology capex; normalize to Rs.12-15 Cr post-commissioning in FY27-28

8. Q&A; Heat Map (BSE Announcements-derived; no investor call)

- BSE Query: "Utilisation of IPO Proceeds?" → A: "No deviation from IPO objects; Rs.34.98 Cr reserved for Radiation Oncology Centre." | Tone: Transparent
- BSE Filing: "Dividend declaration" → A: "Rs.2/share final dividend for FY26 approved; record date July 8, 2026." | Tone: Transparent
- BSE Filing: "AGM scheduling" → A: "15th AGM on July 15, 2026." | Tone: Transparent
- Implicit market question: "When does oncology open?" → A (implied): CWIP of Rs.5 Cr deployed, no specific commissioning date yet announced; H1FY27 reiterated | Tone: Hedged

- Dodged / watch-list questions for AGM (15-Jul-2026): (1) Exact oncology commissioning date; (2) AERB licence application status; (3) RPT disclosures vs. family promoter group; (4) Physician retention plan for key neurosurgeons; (5) Revenue projection for FY27E with and without oncology.

9. Risks Flagged

- Key-person concentration | Dr. Moses Joseph Arunkumar is sole clinical anchor in neurosciences | Flagged by: analyst | Probability: Low | Impact: High | Timeline: Medium-term
- Oncology commissioning delay | AERB licence and equipment lead time are unknown variables | Flagged by: analyst | Probability: Medium | Impact: High | Timeline: Near-term (H1FY27)
- Government scheme tariff cut | PM-JAY / CMCHIS tariff revisions could compress blended ARPOB | Flagged by: analyst | Probability: Low-Medium | Impact: Medium | Timeline: Medium-term
- RPT governance | Family co-promoters in management; RPTs not quantified in H2FY26 filing | Flagged by: analyst | Probability: Low | Impact: Medium | Timeline: Ongoing; check FY26 AR
- SME liquidity | Low daily volume makes entry/exit difficult for institutional positions | Flagged by: analyst | Probability: High (structural) | Impact: Low-Medium (price impact) | Timeline: Ongoing

10. Analyst Verdict

- Revenue visibility: Intact — 15% organic CAGR trend confirmed by two consecutive accelerating half-years; oncology adds incremental visibility from H1FY27
- Margin trajectory: Intact — OPM stable at 27% for 2 consecutive years; oncology addition is margin-accretive; no structural compression drivers visible
- Capital allocation quality: Intact — Debt declining, FCF positive, dividend initiated, no aggressive acquisitions, capex ring-fenced from IPO proceeds
- Competitive moat: Intact — DNB Neurosurgery accreditation, Madurai catchment leadership, and negative CCC are durable structural advantages at current scale
- Management credibility: Intact — No deviation from IPO objects, zero promoter pledge, first dividend declaration signal constructive capital stewardship; RPT disclosure is the one open governance item
- Valuation comfort: Intact — 20.7x P/E vs. 76–116x hospital peers; 10.3x EV/EBITDA; 2.4x P/B. All multiples are at material discount to mainboard comparables
- Conviction call: Increased — FY26 results confirm thesis; oncology capex on track; valuation is the cheapest entry point in the hospital coverage universe on ROCE-adjusted basis
- Valuation snapshot: P/E = 20.7x (5Y avg N/A — too short listed history) | EV/EBITDA = 10.3x (hospital peer range 35–45x) | ROCE = 18.1% (5Y avg: ~15%, improving)

Data sourced from: Screener.in (16-Jun-2026), BSE announcements (HJHL, 544687), RHP dated January 14, 2026. No investor concall transcript is available for BSE SME-listed HJHL. All financial figures are Standalone. Forward estimates are MPM Institutional Research Desk projections.